

Architecture Decisions and Open Questions

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Status: Draft decision register

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1. How to use this document

Important decisions should be recorded before implementation when they affect cost, safety, privacy, user outcomes, or long-term architecture.

Each decision includes:

- Context.
- Options.
- Decision.
- Reason.
- Consequences.
- Review trigger.

ADR-0001 — Content-first implementation

Context

Advanced page physics and WebGL are attractive but do not prove that users understand health information.

Options

1. Build full visual experience first.
2. Build linear content prototype first.
3. Build both in parallel.

Decision

Build and test three linear lessons before advanced physics.

Reason

This creates early evidence of comprehension and reduces the risk of polishing the wrong experience.

Consequences

- Early prototype is less visually spectacular.
- Product value can be validated sooner.
- WebGL work moves off the critical path.

ADR-0002 — Dual reader modes

Decision

Magazine mode and linear mode are both first-class experiences.

Reason

Supports accessibility, low-end devices, screen readers, and users who prefer simple reading.

ADR-0003 — SVG-first educational visuals

Decision

Use semantic SVG for educational diagrams and targetable explanation scenes.

Reason

SVG scales well, supports stable target IDs, can be animated, and is easier to provide with alternatives than canvas-only content.

ADR-0004 — WebGL progressive enhancement

Decision

WebGL enhances selected scenes and page realism but never carries essential information.

Reason

Protects compatibility, performance, accessibility, and battery life.

ADR-0005 — Anonymous-by-default privacy

Decision

Public reading requires no account.

Reason

Sensitive health education should not create unnecessary identity records or browsing history.

ADR-0006 — Versioned content

Decision

Every publication is immutable and versioned, with rollback support.

Reason

Clinical guidance changes and content must be auditable.

ADR-0007 — Human-approved medical content

Decision

AI can assist drafting and engineering but cannot approve or publish medical content.

Reason

Clinical accountability and local-language quality require human review.

ADR-0008 — Git-based content foundation

Decision

Begin with schema-validated content in a version-controlled repository, adding a CMS after the content model stabilizes.

Reason

Improves traceability and reduces premature CMS complexity.

ADR-0009 — Separate technical and sensitive analytics

Decision

Collect technical reliability metrics while minimizing or excluding identity-linked health-topic behaviour.

Reason

The product needs reliability visibility but not sensitive user profiling.

2. Open product questions

- Which country is the first launch scope?
- Which languages are needed first?
- Which age groups are included?
- Is the product individual, facilitator-led, or both?
- Which help and referral services are available?
- Is optional login needed after pilot validation?
- What offline depth is required?
- Who owns clinical approval?
- What is the re-review interval?

3. Open clinical questions

- Which STI topics are in the first edition?
- Which local guidance applies?
- Are any topics age-restricted?
- What urgent-care wording is required?
- What treatment details are safe for general education?
- What referral information can be kept current?

4. Open legal and safeguarding questions

- Does the audience include minors?
- What safeguarding process is required?
- What consent model applies to any field research?
- What privacy and data-residency requirements apply?
- Are there restrictions on health content or referrals in the launch region?
- What asset licenses and image-consent requirements apply?

5. Open technical questions

- Supabase hosting and region.
- Storage bucket structure.
- Offline cache size.
- Target low-end devices.
- WebGL performance budget.

- Audio codec support.
- Analytics provider and retention.
- Error monitoring provider.
- CMS timing.

6. Decision template

```
Decision ID:  
Date:  
Owner:  
Context:  
Options:  
Chosen option:  
Reason:  
Trade-offs:  
Security impact:  
Accessibility impact:  
Clinical/content impact:  
Cost impact:  
Review trigger:  
Approver:
```

7. Decision acceptance rule

No material decision should remain implicit if it affects:

- Medical safety.
- User privacy.
- Data access.
- Launch language.
- Device support.
- Content publication.
- Irreversible vendor or infrastructure choice.